
Endometrial Cancer Early Detection, Diagnosis, and Staging

Know the signs and symptoms of endometrial cancer. Find out how endometrial cancer is tested for, diagnosed, and staged.

Detection and Diagnosis

Finding cancer early, when it's small and hasn't spread, often allows for more treatment options. Some early cancers may have signs and symptoms that can be noticed, but that's not always the case.

- [Can Endometrial Cancer Be Found Early?](#)
- [Signs and Symptoms of Endometrial Cancer](#)
- [Tests for Endometrial Cancer](#)

Stages and Outlook (Prognosis)

After a cancer diagnosis, staging provides important information about the amount of cancer in the body, the best treatment options, and the likely response to treatment.

- [Endometrial Cancer Stages](#)
- [Survival Rates for Endometrial Cancer](#)

Questions to Ask About Endometrial Cancer

Here are some questions you can ask your cancer care team to help you better understand your cancer diagnosis and treatment options.

- [Questions to Ask About Endometrial Cancer](#)

Can Endometrial Cancer Be Found Early?

The best way to find endometrial cancer when it's small (at an early [stage](#)) is to see a health care provider if you have any [signs and symptoms of endometrial cancer](#), such as abnormal vaginal bleeding (that's getting worse, occurring between periods, or happening after menopause) or discharge. Early detection improves the chances that the cancer will be treated successfully.

[Early detection tests for endometrial cancer](#)

[If you are at average risk for endometrial cancer](#)

[If you are at increased risk for endometrial cancer](#)

Early detection tests for endometrial cancer

Early detection (also called **screening**) is the use of tests to find a disease like cancer, in people who don't have symptoms of that disease. Early detection tests can help find some types of cancer at an early stage -- when it's small and hasn't spread beyond the place it started.

If you are at average risk for endometrial cancer

At this time, there are no screening tests or exams to find endometrial cancer early in people at average endometrial cancer risk who have no symptoms.

The American Cancer Society recommends that, at menopause you **should be told about the risks and symptoms of endometrial cancer and strongly encouraged to report any vaginal bleeding, discharge, or spotting to your doctor.**

A pelvic exam can find some cancers, including some advanced uterine cancers, but it's not very useful for finding early endometrial cancers.

Screening tests used for cervical cancer, such as a Pap test or HPV (human papillomavirus) test aren't effective for endometrial cancer. The Pap test, which screens for [cervical cancer](#)¹, can sometimes find early endometrial cancers, but in general, it's not a good test to find endometrial cancer. For information see [Screening Tests for Cervical Cancer](#)².

If you are at increased risk for endometrial cancer

The American Cancer Society recommends that those at increased risk for endometrial cancer be told of their risk and be advised to see their doctor whenever they have any abnormal vaginal bleeding. This includes those at increased risk due to age, late menopause, never giving birth, infertility, obesity, diabetes, high blood pressure, estrogen treatment or tamoxifen therapy.

Also, people with hereditary non-polyposis colon cancer (HNPCC or [Lynch syndrome](#)³) have a very high risk of endometrial cancer.

If several family members have had colon or endometrial cancer, think about getting genetic counseling to learn about your family's risk of HNPCC. If you (or a close relative) have genetic testing and are found to have a mutation in one of the genes for HNPCC, you are at high risk of getting endometrial cancer, as well as other types of cancer. See [Understanding Genetic Testing](#)⁴ for more on this.

The American Cancer Society recommends that anyone **with HNPCC who can get endometrial cancer be offered yearly testing with an endometrial biopsy starting at age 35**. Their doctors should discuss this test with them, including its risks, benefits, and limits. This applies to those known to carry HNPCC-linked gene mutations, those who are likely to carry such a mutation (for instance, when a mutation is known to be present in the family), and those from families with a tendency to develop colon cancer who have not had genetic testing .

Another option for those who have (or may have) HNPCC is to have a hysterectomy once bearing children is no longer a preference. (See [Can Endometrial Cancer Be Prevented?](#))⁵

Hyperlinks

1. www.cancer.org/cancer/types/cervical-cancer.html
2. www.cancer.org/cancer/types/cervical-cancer/detection-diagnosis-staging/screening-tests.html
3. www.cancer.org/cancer/risk-prevention/genetics/family-cancer-syndromes/lynch-syndrome.html
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5. www.cancer.org/cancer/types/endometrial-cancer/causes-risks-prevention/prevention.html

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Signs and Symptoms of Endometrial Cancer

There are symptoms that may point to endometrial cancer. Some are more common as the cancer becomes advanced (grows and spreads).

[Common symptoms of endometrial cancer](#)

[Signs of later-stage endometrial cancer](#)

Common symptoms of endometrial cancer

- Unusual vaginal bleeding
- Vaginal spotting
- Other vaginal discharge

Abnormal vaginal bleeding is the most common symptom of endometrial cancer. This might be a change in periods, bleeding between periods, or bleeding after menopause. Non-cancer problems can also cause abnormal bleeding. But it's important to have a doctor check out any irregular bleeding right away.

For those who have gone through menopause, it's especially important to report any vaginal bleeding, spotting, or abnormal discharge to your doctor. Non-bloody vaginal discharge may also be a sign of endometrial cancer. Even if you can't see blood in the discharge, it doesn't mean there's no cancer. Any abnormal discharge should be checked out by a doctor.

Signs of later-stage endometrial cancer

- Pelvic discomfort
- Weight loss

Pain or a feeling of fullness in the pelvis and losing weight without trying can also be symptoms of endometrial cancer. These symptoms are more common in later stages of the disease. A delay in seeking medical help may allow the disease to worsen and lower the chance of [treatment](#)¹ being successful.

Although any of these signs or symptoms can be caused by things other than cancer, it's important to have them [checked out by a doctor](#).

Hyperlinks

1. www.cancer.org/cancer/types/endometrial-cancer/treating.html

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Tests for Endometrial Cancer

If your regular health care provider believes there's a possibility you could have endometrial cancer, you should be examined by a gynecologist (a doctor trained to diagnose and treat diseases of the female reproductive system). Gynecologists can diagnose endometrial cancer and sometimes treat it. Specialists who treat cancers of the endometrium and other female reproductive organs are called **gynecologic oncologists**.

[Medical history and physical exam](#)

[Using ultrasound to diagnose endometrial cancer](#)

[Endometrial tissue sampling](#)

[Tests on endometrial tissue samples](#)

[Tests to look for endometrial cancer spread](#)

[Blood tests used in endometrial cancer](#)

Medical history and physical exam

If you have any of the [symptoms of endometrial cancer](#), you should see a doctor right away. The doctor will ask about your symptoms, [risk factors](#)¹, and medical history. The doctor will also examine you and give you a pelvic exam feeling the uterus, vagina, ovaries, and rectum to check for any unusual findings. A Pap test, often done with a pelvic examination, is primarily used to screen for cervical cancer. Sometimes, the Pap test may find abnormal glandular cells caused by endometrial cancer.

Using ultrasound to diagnose endometrial cancer

Ultrasound is often one of the first tests used to look at the uterus, ovaries, and fallopian tubes in those with possible gynecologic problems. Ultrasound uses sound waves to take pictures of the inside of the body. A small wand (called a **transducer** or **probe**) gives off sound waves and picks up the echoes as they bounce off the organs. A computer translates the echoes into pictures.

For a **pelvic ultrasound**, the transducer is moved over the skin of the lower part of the belly (abdomen). Often, to get good pictures of the uterus, ovaries, and fallopian tubes, the bladder needs to be full. That's why if you are having a pelvic ultrasound you will be asked to drink lots of water before the test.

A **transvaginal ultrasound** (TVUS) is often a better method of looking at the uterus. For this test, the TVUS probe (that works the same way as the ultrasound transducer) is put into the vagina. Images from the TVUS can be used to see if the uterus contains a mass (tumor), or if the endometrium is thicker than usual, which can be a sign of endometrial cancer. It may also help see if cancer is growing into the muscle layer of the uterus (myometrium). During the TVUS, salt water (saline) may be put into the uterus to help the doctor see the uterine lining more clearly. This technique is called **saline infusion sonohysterography**.

Endometrial tissue sampling

To find out exactly what kind of endometrial change is present, the doctor must take out some tissue so that it can be tested and looked at with a microscope. Endometrial tissue can be removed by endometrial biopsy or by dilation and curettage (D&C) with or without a hysteroscopy. A gynecologist usually does these procedures, which are described below.

Endometrial biopsy

An endometrial biopsy is the most used diagnostic test for endometrial cancer and is very accurate for those who have gone through menopause. It can be done in the doctor's office. A very thin, flexible tube is put into the uterus through the cervix. Then, using suction, a small amount of endometrium is removed through the tube. The suctioning takes about a minute or less. The discomfort is a lot like menstrual cramps and can be helped by taking a nonsteroidal anti-inflammatory drug (like ibuprofen) before the procedure. Sometimes a thin needle is used to inject numbing medicine (local anesthetic) into the cervix just before the procedure to help reduce the pain.

Using hysteroscopy to diagnose endometrial cancer

For this procedure, the doctor puts a tiny telescope (about $\frac{1}{6}$ inch in diameter) into the uterus through the cervix. To get a better view of the inside (lining) of the uterus, the uterus is filled with salt water (saline). This lets the doctor look for anything abnormal, such as cancer or a polyp. This procedure may be done in the office using local anesthesia (numbing medicine) while the patient is awake.

Dilation and curettage (D&C) for endometrial cancer

If the endometrial biopsy sample doesn't provide enough tissue, or if the biopsy suggests cancer but the results are unclear, a D&C must be done. This procedure uses instruments to enlarge (dilate) the cervical canal in order to access the uterus. Another instrument is then used to scrape (curettage) tissue from inside the uterus. This may be done with or without a hysteroscopy.

A D&C is a short outpatient procedure. It can be done while you are under a level of anesthesia determined by your health care team, based on factors such as your health and your preferences.

Tests on endometrial tissue samples

Endometrial tissue samples removed by biopsy or D&C are looked at with a microscope to see if cancer is present. If cancer is found, the lab report will usually include a description of the type of cancer and the grade of the cancer. The report may also include the results of the initial screening test for [Lynch syndrome](#)² and additional molecular classifications.

For more information about the classification of endometrial cancer, see [What Is Endometrial Cancer?](#)³

Testing endometrial tissue samples for gene changes

Tumor cells can be tested for protein and gene changes that will identify those at increased risk for having [Lynch syndrome](#)⁴ and will identify molecular characteristics of the tumor that will guide treatment. These changes include:

- Having fewer mismatch repair (MMR) proteins
- Defects in mismatch repair genes (dMMR)
- DNA changes (high levels of microsatellite instability, or MSI-H) that can happen when one of the genes that causes Lynch syndrome is faulty

If these protein or DNA changes are present, a doctor may recommend [genetic testing](#)⁵ for the genes that cause Lynch syndrome.

The cancer cells might also be tested to see if treatment with [immunotherapy](#)⁶ might be an option, especially for more advanced endometrial cancers.

Tests to look for endometrial cancer spread

If the doctor suspects that your cancer is advanced, you'll probably have to have other tests to look for cancer spread.

Chest x-ray

You might have an [x-ray](#)⁷ of your chest to see if cancer has spread to your lungs.

Computed tomography (CT)

The [CT scan](#)⁸ is an x-ray procedure that creates detailed, cross-sectional images of the inside of your body. CT scans are not used to diagnose endometrial cancer. But they can help see if the cancer has spread to other organs and to see if it has come back after treatment.

Magnetic resonance imaging (MRI)

[MRI scans](#)⁹ are very helpful for looking at the brain and spinal cord. Some doctors also think MRI is a good way to tell whether, and how far, the endometrial cancer has grown into the body of the uterus. MRI scans may also help find enlarged lymph nodes with a special technique that uses very tiny particles of iron oxide. These are injected via a vein and settle into lymph nodes where they can be spotted by MRI.

Positron emission tomography (PET)

In this test, radioactive glucose (sugar) is given to look for cancer cells. Because cancers use glucose (sugar) at a higher rate than normal tissues, the radioactivity will tend to collect in the cancer. A scanner can spot the radioactive deposits. This test can be helpful for spotting small collections of cancer cells. Special scanners combine a [PET scan](#)¹⁰ with a CT to more precisely locate areas of cancer spread. PET scans are not a routine part of the work-up of early endometrial cancer but may be used for more advanced cases.

Cystoscopy and proctoscopy

If a patient has problems that suggest the cancer has spread to the bladder or rectum, the inside of these organs will probably be looked at through a lighted tube. In **cystoscopy**, the tube is put into the bladder through the urethra. In **proctoscopy**, the tube is put in the rectum. These exams allow the doctor to look for cancer. Small tissue samples can also be removed during these procedures for testing. They can be done using a local anesthetic, but some patients may need general anesthesia. Your doctor will let you know what to expect before and after these tests. These procedures were used more in the past, but now are rarely part of the work up for endometrial cancer.

Blood tests used in endometrial cancer

Complete blood count

The complete blood count (CBC) measures red blood cells, the white blood cells and the platelets. Endometrial cancer can cause bleeding, which can lead to low red blood cell counts ([anemia¹¹](#)).

CA-125 blood test

CA-125 is a substance released into the bloodstream by many, but not all, endometrial and ovarian cancers. This test however is not used to diagnose endometrial cancer. Some doctors use CA-125 to help understand if the treatment is working. For example, the level of CA 125 may be high before surgery or any other treatment and might decrease after the treatment.

Hyperlinks

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Endometrial Cancer Stages

After being diagnosed with endometrial cancer, doctors will try to figure out if it has spread and, if so, how far. This process is called **staging**. The stage of a cancer describes how much and where cancer is in the body. Knowing how advanced the cancer is helps determine [how best to treat it](#)¹. **The stage is one of the most important factors in deciding how to treat the cancer and determining how successful the treatment might be.**

How is the endometrial cancer stage determined?

Endometrial cancer grade

Doctors use diagnostic tests to determine the cancer's stage. They will need information based on samples of tissue collected during surgery. Because of this, staging may not be complete until all of the tests are finished and the surgery to remove the tumor has been done.

Endometrial cancer stages range from stage I (1) through IV (4). As a rule, the lower the number, the less the cancer has spread. A higher number, such as stage IV, means cancer has spread to other parts of the body. And within a stage, an earlier letter means a lower stage. Although each person's cancer is unique, cancers with similar stages tend to have a similar outlook and are often treated in much the same way.

How is the endometrial cancer stage determined?

The 2 systems used for staging endometrial cancer are the **FIGO (International Federation of Gynecology and Obstetrics) system** and the **American Joint Committee on Cancer TNM staging system**.

They both stage (classify) this cancer based on 3 factors:

- The extent (size) of the **tumor (T)**: How far has the cancer grown into the uterus? Has the cancer reached nearby structures or organs?
- The spread to nearby **lymph nodes (N)**: Has the cancer spread to the para-aortic lymph nodes? These are the lymph nodes in the pelvis or around the aorta (the main artery that runs from the heart down the back of the abdomen and pelvis).
- The spread (**metastasis**) to distant sites (**M**): Has the cancer spread to lymph nodes or organs in other parts of the body?

Numbers or letters after T, N, and M give more details about each of these factors. Higher numbers mean the cancer is more advanced. Once a person's T, N, and M categories have been determined, this information is combined in a process called **stage grouping** to assign an overall stage.

The staging system in the table below uses the **pathologic stage**. It's determined by examining tissue removed during an operation. This is also known as **surgical staging**. Sometimes, if surgery isn't possible right away, the cancer will be given a **clinical stage** instead. The clinical stage is based on the results of a physical exam, biopsy, and imaging tests done before surgery. For more details, see [Cancer Staging²](#).

The FIGO staging system was updated in 2023 to reflect that some types of endometrial cancer are more aggressive than others.

Endometrial cancer staging can be complex, so ask your doctor to explain it to you in a way you understand.

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Stage	Stage grouping	FIGO Stage	Stage description*
I	T1 N0 M0	II	The tumor is growing inside the uterus and/or is limited to the uterus and ovary for non-aggressive types. The cancer has not spread to nearby lymph nodes (N0) or to distant parts of the body (M0).
			The tumor is limited to the endometrium (inner lining of the uterus) [IA1]. Or

IA	T1a N0 M0	IA	The tumor is a non-aggressive histologic type with growth less than halfway through the underlying muscle layer of the uterus (the myometrium) with no or focal lymphovascular space involvement (LVSI) ** [IA2]. Or The tumor is a non-aggressive histologic type limited to the uterus and ovary [IA3]. The cancer has not spread to nearby lymph nodes (N0) or to distant parts of the body (M0).
IB	T1b N0 M0	IB	Non-aggressive histologic types with growth more than halfway through the myometrium, but not beyond the body of the uterus with no or focal LVSI [IB] . The cancer has not spread to nearby lymph nodes (N0) or to distant parts of the body (M0).
IC	IC	Aggressive histologic types limited to the endometrium.	
II	T2 N0	II	The cancer has spread from the body of the uterus and is growing into the supporting connective tissue of the cervix (called the cervical stroma). Or There is substantial LVSI. Or

	M0		The cancer is an aggressive histologic type with any invasion into the myometrium. The cancer has not spread to nearby lymph nodes (N0) or to distant parts of the body (M0).
IIA	IIA	Non-aggressive histologic type with invasion of the cervical stroma	
IIB	IIB	Non-aggressive histologic type with substantial LVSI	
IIC	T2 N0 M0	IIC	Aggressive histologic type with any myometrial invasion
III	T3 N0 M0	III	The cancer has spread outside the uterus, but has not spread to the inner lining of the rectum or urinary bladder (T3).
IIIA	T3a N0 M0	IIIA	The tumor has spread to the ovary or fallopian tube (and criteria for stage IA3 is not met) [IIIA1] The tumor has spread to the outer surface of the uterus (called the serosa) [IIIA2]. The cancer has not spread to nearby lymph nodes (N0) or to distant parts of the body (M0).

IIIB	T3b N0 M0	IIIB	The cancer has spread to the vagina or to the tissues around the uterus (the parametrium). [IIIB1] The cancer has spread to the pelvic peritoneum (The peritoneum is a sheet of smooth tissue that lines the abdomen and pelvis). [IIIB2] It has not spread to nearby lymph nodes (N0) or to distant parts of the body (M0).
IIIC1	T1-T3 N1, N1mi or N1a M0	IIIC1	The cancer has spread to pelvic lymph nodes (N1, N1mi, or N1a), but not to lymph nodes around the aorta or distant parts of the body (M0). IIIC1i Micrometastasis+ IIIC1ii Macrometastasis^
IIIC2	T1-T3 N2, N2mi or N2a M0	IIIC2	The cancer has spread to lymph nodes around the aorta (para-aortic lymph nodes) (N2, N2mi, or N2a), but not to distant parts of the body (M0). IIIC2i The cancer has spread but is very small. It is a micrometastasis. IIIC2ii The cancer has spread but is very small. It is a macrometastasis.
IVA	T4 Any N M0	The cancer has spread to the inner lining of the rectum or urinary bladder (called the mucosa) (T4). It may or may not have spread to nearby lymph nodes (Any N),	

		but has not spread to distant parts of the body (M0).	
IVB	Any T Any N M1	IVB	The cancer has spread to the abdominal peritoneum. The cancer can be any size (Any T) and it might or might not have spread to other lymph nodes (Any N).
IVC	IVC	The cancer has spread to distant parts of the body such as lungs, liver, brain or bone or has spread to lymph nodes outside of the abdomen or lymph nodes above the kidneys.	

*The following additional categories are not listed on the table above:

- **TX:** Main tumor cannot be assessed due to lack of information.
- **T0:** No evidence of a primary tumor.
- **NX:** Regional lymph nodes cannot be assessed due to lack of information.

A pathology report may also indicate isolated tumor cells (ITCs). This is noted when small amounts of tumor (<0.2mm and <200 cells) are found in the lymph nodes. The significance of this finding is unknown at this time, but it will be noted as pN0(i+).

**Lymphovascular space involvement (LVSI) = tumor cells are found around or near blood vessels or the lymph system.

+Micrometastasis = 0.2-2mm and/or >200 cells.

^Macrometastasis = larger than 2 mm.

Endometrial cancer grade

Doctors can also describe this type of cancer by its grade (G). The grade describes how closely cancer cells look like healthy cells when seen with a microscope.

The doctor compares the cancerous tissue with healthy tissue. Healthy tissue usually has many different types of cells grouped together.

- If the cancer looks similar to healthy tissue and has different cell groupings, it is called a **differentiated** or **low-grade tumor**.
- If the cancerous tissue looks very different from healthy tissue, it is called a **poorly differentiated** or **high-grade tumor**.

The cancer's grade may help the doctor predict how quickly the cancer will grow. In general, the slower the growth, the better the prognosis.

The letter "G" is used to define a grade for uterine cancer.

GX: The grade cannot be evaluated.

G1: The cells are well differentiated.

G2: The cells are moderately differentiated.

G3: The cells are poorly differentiated.

Hyperlinks

1. www.cancer.org/cancer/types/endometrial-cancer/treating/by-stage.html
2. www.cancer.org/cancer/diagnosis-staging/staging.html

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Survival Rates for Endometrial Cancer

Survival rates can give you an idea of what percentage of people with the same type and stage of cancer are still alive a certain amount of time (usually 5 years) after they were diagnosed. They can't tell you how long you will live, but they may help give you a better understanding of how likely it is that your treatment will be successful.

Today in the United States, there are more than 600,000 uterine cancer survivors.

[What is a 5-year relative survival rate?](#)

[Where do these numbers come from?](#)

[Five-year relative survival rates for endometrial cancer](#)

Keep in mind that survival rates are estimates and are often based on previous outcomes of large numbers of people who had a specific cancer, but they can't predict what will happen in any particular person's case. These statistics can be confusing and may lead you to have more questions. Ask your doctor how these numbers might apply to you.

What is a 5-year relative survival rate?

A **relative survival rate** compares women with the same type and stage of endometrial cancer to women in the overall population. For example, if the **5-year relative survival rate** for a specific stage of endometrial cancer is 90%, it means that women who have that cancer are, on average, about 90% as likely as women who don't have that cancer to live for at least 5 years after being diagnosed.

Where do these numbers come from?

The American Cancer Society relies on information from the Surveillance, Epidemiology, and End Results (SEER) database, maintained by the National Cancer Institute (NCI), to provide survival statistics for different types of cancer.

The SEER database tracks 5-year relative survival rates for endometrial cancer in the United States, based on how far the cancer has spread. The SEER database, however, does not group cancers by [FIGO or AJCC TNM stages](#) (stage 1, stage 2, stage 3, etc.). Instead, it groups cancers into localized, regional, and distant stages:

- **Localized:** There is no sign that the cancer has spread outside of the uterus.
- **Regional:** The cancer has spread from the uterus to nearby structures or lymph nodes.
- **Distant:** The cancer has spread to distant parts of the body such as the lungs, liver or bones.

Five-year relative survival rates for endometrial cancer

(These numbers are based on people diagnosed with endometrial cancer between 2015 and 2021.)

SEER* Stage	5-year Relative Survival Rate
Localized	96%
Regional	72%
Distant	22%
All SEER stages combined	84%

*SEER= Surveillance, Epidemiology, and End Results

Understanding the numbers

- **These numbers apply only to the stage of the cancer when it is first diagnosed.** They do not apply later on if the cancer grows, spreads, or comes back after treatment.
- **These numbers don't take everything into account.** Survival rates are grouped based on how far the cancer has spread, but your age, overall health, how well the cancer responds to treatment, and other factors will also affect your outlook.
- **Patients now being diagnosed with endometrial cancer may have a better outlook than these numbers show.** Treatments improve over time, and these numbers are based on those who were diagnosed and treated at least five years earlier.

References

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Questions to Ask About Endometrial Cancer

As you cope with cancer, you need to have honest, open talks with your doctor. You should feel comfortable asking any question, no matter how small it might seem. Nurses, social workers, and other members of your treatment team might also be able to answer many of your questions. Here are some examples of questions you might want to ask:

When you're told you have endometrial cancer

When deciding on a treatment plan

During treatment

After treatment

Remembering what the doctor says

When you're told you have endometrial cancer

- What [type and grade of endometrial cancer](#)¹ do I have?
- Has my cancer spread outside the uterus?
- What is the [stage](#) of the cancer and what does that mean?

When deciding on a treatment plan

- What [treatments](#)² might be right for me? What do you recommend? Why?
- Are there [clinical trials](#)³ that I should think about?
- What's the goal of my treatment?
- What should I do to be ready for treatment?
- What risks or [side effects](#)⁴ should I expect? (Ask about both short- and long-term side effects.)
- Will treatment affect my sex life?
- Will I be able to have children after treatment?
- What are the chances my cancer will come back (recur) with the treatments we have discussed?
- Will my health insurance cover treatment? How much will I have to pay?

During treatment

Once treatment starts, you'll need to know what to expect and what to look for. Not all of these questions may apply to you, but asking the ones that do may be helpful.

- How will we know if the treatment is working?
- How will you monitor my response to treatment?
- Is there anything I can do to help prevent or manage side effects?
- What symptoms or side effects should I tell you about right away?
- How can I reach you on nights, holidays, or weekends?
- Will I need to change what I eat during treatment?
- Are there any limits on what I can do?
- What kind of exercise should I do, and how often?
- Can I still take hormone replacement therapy?
- How will I feel during treatment?

After treatment

- Are there any limits on what I can do?
- What kind of exercise should I do now?
- What type of follow-up will I need after treatment?
- How often will I need to have follow-up exams and imaging tests?
- Will I need any blood tests?
- How will we know if the cancer has come back? What symptoms should I watch for?
- What will my options be if the cancer comes back?
- When can I go back to my usual activities at work and/or around the house?

Along with these sample questions, be sure to come up with some of your own. For instance, you might want to ask about [getting a second opinion](#)⁵, or you may need specific information about how long it might take you to recover so you can plan your work schedule.

Remembering what the doctor says

It's hard to remember all of the things you're told at each doctor's visit, especially when you're anxious or afraid.

Even if the doctor carefully explains things, you might not hear or remember all that's said or shown to you. Here are some ways to help you remember everything your doctor tells you.

- Take notes on what your doctor says.
- Ask if you can record your talks.
- Take a family member or friend with you. Ask them to remind you of questions you want to ask and ask them to take notes, so you can focus on listening and talking directly with the doctor.

Doctors aren't the only ones who can give you information. Other health care professionals, such as nurses and social workers, can answer a lot of your questions. Learn more in [Who Is the Cancer Care Team?](#)⁶

Hyperlinks

1. www.cancer.org/cancer/types/endometrial-cancer/about/what-is-endometrial-cancer.html
2. www.cancer.org/cancer/types/endometrial-cancer/treating.html
3. www.cancer.org/cancer/managing-cancer/making-treatment-decisions/clinical-trials.html
4. www.cancer.org/cancer/managing-cancer/side-effects.html
5. www.cancer.org/cancer/managing-cancer/making-treatment-decisions/seeking-a-second-opinion.html
6. www.cancer.org/cancer/preparing-for-treatment/cancer-care-team.html

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Written by

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